KONGPOSH IMAGING CENTRE
128-slice CT | 3T MRI | Ultrasound | Digital Radiography
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Reg. No. JK/SGR/DC/2018/0094

CECT ABDOMEN & PELVIS (TRIPHASIC) WITH ORAL CONTRAST

Patient:	**GH. NABI DAR**	Study ID:	**KIC/CT/26/7741**
Age / Sex:	67 Y / M	Dated:	**04/07/2026**
Referring:	Dr. Showkat Ahmad Mir (Surgical Oncology)	Contrast:	Iohexol 80 mL IV, uneventful

Clinical details: Epigastric fullness, early satiety and vomiting x 2 months; weight loss. UGI endoscopic biopsy (HPE dated 26.06.2026): adenocarcinoma, antrum.

Findings

- Stomach: circumferential, enhancing, irregular wall thickening of the gastric antrum and distal body (maximum thickness 18 mm, length about 6 cm) with loss of the fat plane with the pancreatic head at one point; perigastric fat stranding.
- Lymph nodes: multiple enlarged perigastric nodes along the lesser curvature (largest 14 x 11 mm) and a 12 mm node at station 8a. No para-aortic nodes.
- Liver normal in size with no focal lesion. Gall bladder, pancreas, spleen, both kidneys and adrenals unremarkable.
- No ascites, omental or peritoneal deposits. Visualised lung bases clear. No lytic or sclerotic bony lesion.

OPINION

Circumferential antral and distal body wall thickening with perigastric and station 8a lymphadenopathy, in keeping with the known gastric carcinoma. No evidence of hepatic, peritoneal or distant metastasis.

Dr. Rubina Qadri
M.D. (Radiodiagnosis)
Reg. No. JKMC-6841
Consultant Radiologist

*** End of Report ***

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